C. Cardiovascular Risk Assessment and Management in Psoriasis

C.3. A baseline assessment of lipid levels should be performed in patients with psoriasis and repeated according to national guidelines. A more frequent assessment of lipids should be carried out in patients with moderate to severe psoriasis.

C.4. Total cardiovascular risk estimation using a risk assessment system such as SCORE2 is recommended for asymptomatic adults with psoriasis who do not have evidence of cardiovascular disease, diabetes mellitus, chronic kidney disease, familial hypercholesterolemia, or LDL $>$ 190 mg/dL.

C.5. Dermatologists should inform patients about the association between cardiovascular disease and psoriasis.

C.6. Lifestyle interventions such as counseling on diet, smoking cessation, and exercise are very important for patients with psoriasis.

C.7. Body Mass Index, blood pressure, cholesterol levels (LDL-C), and hemoglobin A1c should be assessed periodically in patients with psoriasis.

C.8. There should be a focus on educating both patients and healthcare providers about cardiovascular risk and lipid management in psoriasis.

D. Non-invasive Cardiovascular Imaging

D.1. Assessment of arterial (carotid and/or femoral) plaque burden using vascular ultrasonography may be considered as a risk modifier in individuals with psoriasis who are at low or moderate cardiovascular risk.

D.2. Coronary artery calcium (CAC) scoring using CT may be considered as a risk modifier in the cardiovascular risk assessment of asymptomatic individuals with psoriasis who are at low or moderate risk.

D.3. Routine use of other vascular tests or imaging methods (beyond CAC scoring or ultrasound for plaque detection) is not recommended.

E. LDL-C Thresholds (see Figure 3 for cardiovascular risk levels)

E.1. LDL-C levels in psoriasis patients with low to moderate cardiovascular risk should be < 100 mg/dL (2.6 mmol/L).

E.2. LDL-C levels in psoriasis patients with high cardiovascular risk should be < 70 mg/dL (1.8 mmol/L), with a $\geq 50\%$ reduction from baseline LDL-C.

E.3. LDL-C levels in psoriasis patients with very high cardiovascular risk should be < 55 mg/dL (1.4 mmol/L), with a $\geq 50\%$ reduction from baseline LDL-C.

F. Treatment with Statins

F.1. High-intensity statin therapy should be prescribed when the treatment goal is at least a 50% reduction in LDL-C (e.g., atorvastatin 40–80 mg daily; rosuvastatin 20–40 mg daily).

F.2. Moderate-intensity statin therapy should be prescribed when the goal is a 30% to 49% reduction in LDL-C (e.g., atorvastatin 10–20 mg daily; fluvastatin 80 mg daily; lovastatin 40–80 mg; pitavastatin 1–4 mg; pravastatin 40–80 mg; rosuvastatin 5–10 mg; simvastatin 20–40 mg).

F.3. It is recommended to use high-intensity statin therapy up to the highest tolerated dose to achieve the specific risk-based LDL-C target.

F.4. The optimal statin regimen in psoriasis is not definitively known, but statins with strong anti-inflammatory effects (e.g., atorvastatin or rosuvastatin) may be particularly beneficial.

F.5. In patients aged 20–75 years with severe hypercholesterolemia (LDL-C $\geq$ 190 mg/dL), initiate high-intensity statin therapy immediately, regardless of 10-year atherosclerotic cardiovascular disease (ASCVD) risk.

F.6. In psoriasis patients with diabetes, moderate- or high-intensity statin therapy should be initiated to achieve the LDL-C target appropriate for their specific cardiovascular risk category.